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The Honorable Dr. Mehmet Oz
Administrator
Centers for Medicare & Medicaid Services
Department of Health and Human Services
Attention: CMS-0042-NC
P.O. Box 8013
Baltimore, MD 21244-8013

***Delivered Electronically***

**RE: Request for Information; Health Technology Ecosystem, CMS–0042–NC [RIN 0938-AV68]**

Dear Administrator Oz:

On behalf of Flatiron Health, Inc. ("Flatiron," "us," "we," "our"), we appreciate the opportunity to respond to the Request for Information on the Health Technology Ecosystem (the "RFI"). We have provided detailed responses to your questions below from the perspective of a smaller, oncology-focused certified EHR developer, which broadly fall into the following themes:

- ***Standards-based exchange is the gold standard to promote effective data sharing.*** Standards such as USCDI and FHIR® with their robust implementation frameworks support a health technology ecosystem that advances in alignment with evolving data standards and industry best practices. Flatiron believes data sharing and interoperability are best served with regulations that focus on standards-based exchange, ideally with optionality to reflect the scope of data actually maintained by the certified entity.
- ***Incremental updates to the certification program allow for thoughtful development in parallel with evolving technology standards and support diverse market participation in the certification program:*** The existing Assistant Secretary for Technology Policy/Office of the National Coordinator for Health Information Technology ("ASTP") certification program provides an important baseline level of quality and transparency the provider market relies on. While there is certainly room to streamline the program, neither the introduction of onerous new requirements nor a radical clawback of the program would serve providers, their patients, or the health technology ecosystem writ large. Flatiron encourages ASTP to promote optionality within the certification program by limiting criteria included in the mandatory base EHR definition, which allows developers to invest in certified functionality that is tailored to the needs of their customer base.
- ***Retain existing information blocking exceptions, which balance the laudable goal of broad information sharing with practical limitations.***

Our detailed responses follow below.

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## Background on Flatiron

Flatiron is driven by a shared mission to improve and extend lives by learning from the experience of every person with cancer, and our vision is to build a world where technology and science close the gap between care and research. Flatiron provides healthcare technology products and services to community oncology clinics and academic medical centers across the United States, including an Electronic Health Record ("EHR") and other related services that form the cornerstone of a platform to advance and accelerate cancer research. Flatiron's ASTP/ONC-certified product, OncoEMR®, is an oncology-specific EHR built for community-based practices and has been supporting practices for nearly two decades. Having an oncology-first approach means our EHR is tailored to the needs of oncology providers and their patients, recognizing the distinct treatment, data and research needs for cancer care.

Flatiron supports and shares CMS and ASTP's vision for improved interoperability and data sharing across EHRs and across the healthcare landscape to improve patient care, streamline clinical workflows, and accelerate progress towards a patient-centric health system.

The community oncologist is Flatiron's OncoEMR's primary customer and user. Community oncology care is distinguished from cancer care delivered in other settings by its personalization, convenience, affordability, and the high-quality nature of the care provided. Community oncology care offers oncology patients excellent oncology care close to home without the expense and burden of frequent travel to an academic medical center or large health system.[1]

However, community oncology care faces persistent challenges. There is an acute shortfall of oncologists and cancer care specialists.[2] Community oncology care practices also face unprecedented financial pressure as they navigate Medicare physician payment cuts as well as a shifting reimbursement landscape. Complex value-based models and payer requirements, coupled with escalating drug costs, have placed enormous financial strain on community oncology practices. Finally, the rapid pace of treatment innovation, while thrilling, can be incredibly difficult to keep up with. All of this means oncologists need to do more with less, which increases physician burnout, and can have a long-term negative impact on accessibility of care in the community setting.

Consequently, Flatiron is committed to serving its customers through participation in the ONC Health IT Certification Program ("Certification Program") and has previously provided comments to ASTP.[3] We look forward to continued opportunities to comment on any proposed changes to the Certification Program to ensure the program effectively supports critical policy

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[1]Ambroggi, M. et al. (Dec. 2015), Distance as a Barrier to Cancer Diagnosis and Treatment: Review of the Literature. *Oncology* 20(12), 1378–1385, available at https://www.ncbi.nlm.nih.gov/pmc/articles/PMC4679078/#.
[2]American Society for Clinical Oncology, *Oncology Workforce Shortages* (2023), available at https://society.asco.org/sites/new-www.asco.org/files/content-files/news-initiatives/documents/2023-workforce-brief.pdf.
[3] Flatiron, Comment Letter on Proposed Rule Health Data, Technology, and Interoperability: Patient Engagement, Information Sharing, and Public Health Interoperability (HTI-2) (Oct. 4, 2024), https://www.regulations.gov/comment/HHS-ONC-2024-0010-0199.

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goals such as enhanced interoperability while minimizing regulatory burdens that provide limited benefit to our oncology provider customers.

**Detailed Responses to CMS and ASTP Questions**

*Feedback on the Certification Program [TD-9 (a-c)]*

The federal ONC Health IT Certification Program serves important policy goals and provides a critical source of stability for the EHR industry and its provider customers by:

- Giving health care providers a metric to understand the quality and content of a given health IT product and supporting their participation in value-based care programs. This market transparency allows providers to make informed purchasing decisions, participate in standards-based interoperable information sharing, and provide high quality care to Americans through programs like the Merit-based Incentive Payment System (MIPS);
- Negating the need for individual states to develop inconsistent requirements and/or certification programs, which would create a complex and potentially contradictory regulatory landscape; and
- Supporting cutting edge research both by supporting FDA Part 11 compliance and by ensuring that data from different health systems and EHRs can be exchanged and interpreted consistently, which allows researchers to access larger and more inclusive real-world datasets, thereby facilitating higher quality research and supporting medical product development.

**We do not support "redefining" certification to prioritize API-enabled capabilities exclusively.** The ONC Health IT Certification Program supports many important capabilities outside of the API-focused criteria, especially those related to the privacy and security of electronic health information ("EHI") such as (d)(2) and (d)(3) audit trail capabilities. Audit logs allow providers to appropriately track access to EHI as well as meet their HIPAA compliance obligations. Additionally, ONC-certified audit trails allow EHR data to meet certain FDA Part 11 requirements for electronic data and signatures, which facilitates participation in clinical trials and cutting edge drug development from a broader group of providers.

Additionally, while we support the goals of enhancing interoperability and increasing the opportunities and use cases of data exchange, we encourage CMS and ASTP to take an incremental and flexible approach, ideally with optionality for specialty EHRs, to provide developers time to effectively deploy upgrades that are appropriately tailored to their customers' unique needs and avoid high costs that would be passed on to providers and their patients. Requiring new API and interoperability capabilities involves more than just support for

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new data and an API connection. When developing support for new data elements and API capabilities, EHR developers have to consider how to embed data and data exchange workflows within our existing system and workflows. Designing clinically relevant, safe workflows to support new data classes and interoperability makes development work costly. Additionally, adding many new requirements all at once can be disruptive for providers if forced to implement significant changes overnight. Imposing multiple new, non-discretionary requirements to maintain certification, some of which will quickly become obsolete or are irrelevant for our patient population, impedes our ability to innovate and potentially forces us to pass on additional costs to our customers for features that do not improve their day-to-day ability to care for oncology patients. In the oncology space, this means we will have less time to focus on core patient safety/security functionality and features critical to oncologists and patients with cancer.

**We urge ASTP to make any new API-focused certification requirements optional, rather than part of the base EHR definition to allow developers the ability to invest in the API capabilities that best serve their customers** who have varying needs when it comes to API-based exchange. Additionally, we urge ASTP to provide adequate time of 18-24 months for developers to implement any new API-based capabilities.[4]

*USCDI [TD-7, TD-13]*

In general, Flatiron is supportive of USCDI as a standard vocabulary for data exchange; however, going forward, to the extent ASTP desires to make new versions of USCDI mandatory, it would be helpful to provide **optionality for specialty EHRs**. While USCDI is helpful for ensuring the health technology ecosystem can exchange data using the same vocabulary, not all data elements apply equally to all actors. For example, USCDI v1 had a number of mandatory data elements that do not apply to our providers' adult oncology population such as vital signs related to infants (e.g., Head Occipital-frontal Circumference Percentile). If ASTP ultimately expands the required scope of USCDI, we would encourage the agency to consider how optionality that reflects the reality of the data maintained by specialized certified health IT providers could support a practical expansion of interoperability.

Additionally, while we support the goals of enhancing interoperability and increasing the opportunities and use cases of data exchange, Flatiron would strongly **discourage ASTP from**

[4] The Electronic Health Records Association, which represents a broad coalition of EHR developers, has identified 18-24 months as "necessary for the development of new EHR features once standards for that development have reached sufficient maturity for adoption." *See* HIMSS Electronic Health Record Ass'n, *Feedback to Maryland Health Care Commission on Proposed Revisions to COMAR 10.25.07, 10.25.18, and 10.11.18* (Feb. 12, 2024), https://www.ehra.org/sites/ehra.org/files/EHR%20Association%20Feedback%20to%20Maryland%20on%20COMAR%2010.11.18%2C%2010.25.07%2C%20and%2010.25.18.pdf.

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**shifting from standards-based exchange of USCDI to non-standards based EHI** for FHIR API data exchange requirements. With a defined set of data and mature implementation guides to exchange USCDI data via FHIR APIs and CCDs, USCDI-based requirements have made it easier to integrate with other Health IT systems and are fundamental to promoting interoperability. EHI is not consistently defined, leaving developers and providers to make the decisions of what parts of a record are EHI. Consequently, the format and content of EHI data exchanged through non-standard APIs could vary significantly from system to system. The lack of standardization and mature implementation guides covering the scope of data that must be included for the exchange of EHI would not, on its own, encourage interoperable exchange because the data recipient would not be able to easily ingest data from other systems. In fact, custom development might be required by both sending and receiving systems for each non-standard data source to ensure the data pulled from APIs is properly ingested in the receiving system. Furthermore, EHI is already accessible to users via the EHI export, as unstructured data via FHIR's Document Reference Profile and CCDs, and through other non-certified data exchange mechanisms such as HL7v2 interfaces. While not API-based, these mechanisms, in combination with USCDI API-based data exchange, create much-needed flexibility to exchange EHI without creating additional burden on developers and providers. Requiring EHI data to be exchanged by another mechanism would require extensive development time and cost, which would force EHR developers to cease critical work demanded by customers to serve their patients and force developers to pass on these costs to our cash-strapped customers

Ultimately, while exchanging from "all aspects of the chart" is desirable, we need technology and standards to evolve to support it. Imposing multiple new, non-discretionary requirements to maintain certification, impedes our ability to innovate and potentially forces us to pass on additional costs to our customers for features that do not improve their day-to-day ability to care for oncology patients. Until we reach a point in time where data exchange standards support sharing large unstructured portions of the chart in a meaningful manner, we encourage ASTP to continue to focus on the  incremental, progressive adoption of standards-based data exchange, which currently works well to support an interoperable health technology ecosystem and evolves in lock step with technological advances and expert consensus.

*EHI Export* [TD-11]

Flatiron believes the **EHI export criterion does not need to be changed**. Furthermore, altering the EHI export requirements from their current flexible approach would result in greater additional costs for providers and unnecessary burden on developers without any obvious benefit. The current requirements strike the right balance of providing access to all EHI while allowing developers to set up their exports in a manner that reflects customer needs and

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expectations. However, if ASTP ultimately chooses to change EHI export requirements, we urge ASTP to leverage existing certified technology capabilities.

*Bulk FHIR Quality Data Submission* [TD-9(e)]

Flatiron supports increasing options for data submission to CMS; however, **CMS should not make Bulk FHIR submission of quality data mandatory** as there would be significant costs associated with setting up this method of data submission that ultimately may not serve our customers' data needs. In order to facilitate quality reporting through Bulk FHIR, developers would need to treat each quality measure as a separate integration with a unique patient population that would need to be surfaced in accordance with CMS specifications that change over time. Utilizing Bulk FHIR for quality data submission would also be a stretch for FHIR capabilities which, as RESTful APIs, are not truly intended to exchange the very large amounts of data that are required for value-based care quality reporting but instead are optimized for more frequent exchange of smaller packets of data. Additionally, a direct connection between CMS and the developer for quality reporting would exclude the provider and remove a critical feedback loop for them to adjust their workflows as needed to improve their performance throughout the year. If Bulk FHIR reporting were required, developers would likely need to set up duplicative bulk FHIR integrations for their providers who are unlikely, based on current incentive structures and constrained resources, to have the infrastructure to ingest data sent by Bulk FHIR. In our experience, community oncology providers lack comfort with Bulk FHIR and do not have large IT departments to leverage for implementation assistance; consequently, we have seen limited adoption of Bulk FHIR from our customers. There is also no guarantee that all of the data required for clinical quality data submissions will be immediately available over FHIR based on existing implementation guides, likely requiring additional implementation work by developers to ensure they can correctly surface quality data via FHIR.

Creating these integrations and cohorts would be fairly burdensome both for developers and their provider customers, and would not serve our customers' desire for accessible, real-time tracking of quality metrics to respond to suboptimal performance throughout the year. Consequently, rather than saving effort, if mandated, bulk FHIR would merely require a duplication of effort to maintain multiple systems for the curation of quality data for submission to CMS. Additionally, we expect that our providers who are already facing financial challenges would have to take on additional expenses for IT and other vendors to implement Bulk FHIR.

We do believe CMS could help reduce the burden of setting up bulk FHIR data submission by identifying patient cohorts (i.e. which patients in a given patient population should be queried) from which to collect the requisite quality data via Bulk FHIR without involvement from the developer after initial set up. If CMS could independently identify and maintain the cohorts of

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which patients' data they need for quality reporting, it would help to offset the initial implementation costs of these bulk FHIR connections and support the collection of data in line with CMS intent. Without this support, developers would be required to build and regularly update both lists of patients whose data needs to be included for quality data submission and reports to pull the quality data from EHR ecosystems.

While we have concerns about *mandating* the use of bulk FHIR for quality reporting, we do not have any objections to the creation of an *optional* reporting pathway via bulk FHIR with CMS support that can be adopted by those who find it most valuable for their provider customers.

*Digital Identity* [PR-9, PR-10, PC-14(e)]

Digital identity credential providers such as CLEAR and login.gov have not been heavily utilized in the heath care context, and ASTP should monitor the efficacy of digital identity in the healthcare context prior to adding it into the Health IT Certification program. If ultimately ASTP does integrate digital identity credentials into the ONC Certification Program, we strongly encourage ASTP to make these updates optional. Our customers and their patients have adopted identity verification workflows. Forced workflow changes are disruptive, causing increased administrative burden that may take time away from patient care. Integrating these identity credentials would be costly for developers and take time away from developing features requested by providers for the benefit of their patients. Consequently, **we encourage CMS and ASTP to thoughtfully quantify and weigh any proposed benefit of these programs against the administrative and costly burdens of integrating new identity verification processes into health IT when considering requiring the use of these processes in government regulated health care programs.**

*Information Blocking* [PR-12]

**Flatiron supports ASTP and CMS' goal to create a health technology ecosystem that supports the sharing of health information for the benefit of patients, and we believe the existing information blocking regulations appropriately balance these goals while providing critical exceptions for actors to protect patient privacy and limit their obligations to fulfill infeasible requests.** In general, certified health IT developers like Flatiron must refrain from interfering with the access, exchange, or use of EHI.[5] However, ASTP thoughtfully developed a limited number of exceptions that are "reasonable and necessary to promote competition and consumer welfare."[6] Even as a smaller specialty EHR, Flatiron receives hundreds of requests regarding EHI each year. While we are able to support the

[5] 42 U.S.C. § 300jj-52
[6] 85 Fed. Reg. 25820 (May 5, 2020)

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majority of these requests, we simply could not absorb the cost of accommodating every EHI request in the manner in which it is requested. To operate sustainably, the information blocking exceptions provide critical safeguards for developers that balance the imperatives of health information sharing with practical implementation constraints. While Flatiron is supportive of all existing exceptions, the following exceptions are particularly impactful for Flatiron and our customers:

- *Manner Exception:* The Manner Exception in conjunction with the more recently adopted Manner Exception Exhausted corollary provide an important framework for developers to engage in constructive dialogue with EHI requestors to identify a feasible path for fulfilling EHI requests as well as a safe harbor for terminating such dialogues where alternative manners have been exhausted. Fulfilling a request in the exact manner requested can be infeasible–in these instances, the manner exception allows developers to provide reasonable alternatives in line with their capabilities and resources to satisfy these requests. Where requests can be fulfilled by existing certified capabilities, the manner exhausted exception provides regulatory certainty that terminating protracted negotiations does not pose information blocking risk. By ensuring requestors cannot simply circumvent the important principles of the manner exception by refusing to accept any alternative manners, in HTI-1 ASTP finalized the Manner Exception Exhausted sub-exception to the Infeasibility Exception to prevent the "diversion of resources away from standards-based and scalable manners of exchange" by allowing offering alternative interoperable manners to definitively meet an actor's information blocking obligations.[7]
- *"Infeasibility Under the Circumstances" Exception:* While Flatiron is supportive of a health technology ecosystem that enables the broad sharing of health data, the breadth and amount of EHI requests can be overwhelming for a smaller, specialty EHR developer. The ability to provide a non-discriminatory, appropriately documented rationale as to why fulfilling a request is infeasible under the circumstances for a developer provides an important basis for denying requests that are simply infeasible or impossible to fulfill. Removing this exception would be catastrophic to the financial viability of smaller EHR companies like Flatiron with limited capacity to take on highly customized, non-standard and complex data sharing requests that are not generalizable to benefit our broader customer network.
- *Fees Exception:* Fulfilling requests for EHI outside the scope of typical services offered (e.g., HL7 interfaces, FHIR integrations etc.) can involve unpredictable costs. The fees exception allows developers like Flatiron to recoup those costs within limits to ensure the fees imposed do not discourage the sharing of health information. The Fees Exception is another example of a carefully calibrated balance to allow the EHR industry

[7] 89 Fed. Reg. 1381 (Jan. 9, 2024)

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to remain financially stable while ensuring such fees to not hinder information sharing or serve anti-competitive interests.

- *Preventing Harm and Privacy Exceptions*: Our provider customers regularly rely on the preventing harm and privacy exceptions to comply with state law and protect their patients' wellbeing. Our providers are often forced to deliver some of the most challenging news in healthcare - a patient's cancer diagnosis and prognosis. States such as Kentucky and Tennessee have recognized that certain test results should not be delivered without clinician review to ensure patients receive news of malignancies and other sensitive test results with the appropriate clinical context and compassion that providers are trained to communicate. Other states have focused on limiting the sharing of sensitive health information without appropriate consent. These exceptions, among other things, provide important regulatory clarity to our provider customers that they can temporarily withhold certain sensitive results or other health information in order to comply with state law and/or protect their patients when they are particularly vulnerable.

**We urge ASTP to retain the existing information blocking exceptions, which provide an appropriate balance between encouraging data sharing and maintaining a robust marketplace of financially viable certified health IT.**

Additionally, we encourage ASTP to reconsider the fee conditions in the Conditions and Maintenance of Certification codified at 45 C.F.R. 170.404(a)(3) that go beyond the balanced approach enumerated in the information blocking fees exceptions. The information blocking requirements support fair-market, non-discriminatory pricing while offering pricing model flexibility (e.g., the ability to directly charge applications versus requiring customers to pass-through costs). This is especially true for patient applications that can register and connect to the FHIR server of certified EHR without provider customer knowledge or approval). Compliance with an additional set of pricing regulations is duplicative and costly.

If you have any questions about our comments or need any additional information, please do not hesitate to contact Kate Estep at onc@flatiron.com.

Thank you for your consideration.

Sincerely,

Signed by:

Kate Estep

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**Kate Estep**

Vice President, Head of Product

Flatiron Health